

Cutaneous Lesions

Classically, lesions are divided into two categories: **specific** and **non-specific**. Specific cutaneous lesions contain granulomas on histopathologic examination, whereas non-specific skin findings are reactive and do not exhibit sarcoidal granulomas.

Specific Lesions

Specific sarcoidal lesions most often appear on the head and neck (Figs. 153-2, 153-3, and 153-4) but may occur symmetrically or asymmetrically on any part of the skin and mucosa. Lesions can present in various morphologies, including macules, papules, patches, plaques, and nodules. Scalp involvement may lead to alopecia, and nail changes can also occur. These lesions may be scaly, telangiectatic, or atrophic. Although diverse in appearance, several clinical presentations are classically associated with cutaneous sarcoidosis.

The most common presentation is the **papular form** (see Figs. 153-2, 153-3, and 153-4). These firm, 2- to 5-mm papules typically have a translucent red-brown or yellow-brown hue. The yellow-brown color is often described as resembling "apple jelly," a feature enhanced with diascopy (Fig. 153-5). Diascopy may also reveal the underlying nodular quality of the granulomas. However, this finding is not pathognomonic for sarcoidosis, as other granulomatous conditions such as lupus vulgaris may show similar characteristics. The lesions often have a waxy appearance due to mild epidermal atrophy, though epidermal changes vary. Papular lesions predominantly affect the face and neck, especially the periorbital skin (see eFig. 153-5.1 in online edition).

Over time, papular lesions may coalesce into **annular plaques** (Fig. 153-6), **non-annular plaques** (Fig. 153-7 and see eFig. 153-7.1 in online edition), or **nodules** (Fig. 153-8), which may retain the translucent quality of the original papules.

Lupus pernio refers to symmetric, violaceous, indurated plaques and nodules affecting the nose, earlobes, cheeks, and digits (Figs. 153-9, 153-10, and 153-11). This variant is distinctive and strongly associated with systemic disease, particularly chronic upper respiratory tract involvement. Lesions of lupus pernio may extend into the nasal sinuses and are often progressive and disfiguring.